

HOW TO OPEN AN IN-HOME (NON-MEDICAL) CARE AGENCY in Ohio

An ODH-licensed Nonmedical Home Health Services agency — private pay & Medicaid.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Ohio model — read this first

Here's something that changed recently and that most online guides still get wrong: Ohio NOW requires a state license for non-medical home care. Since October 1, 2022 (under Ohio Revised Code Chapter 3740), the Ohio Department of Health (ODH) licenses home health agencies for BOTH skilled and non-medical services — issuing a Nonmedical Home Health Services License and a Skilled Home Health Services License. A business that sends aides into people's homes to provide hands-on personal care and home-health-aide help needs the Nonmedical license. To then serve Medicaid clients, you also enroll as a Medicaid provider and get certified by the Ohio Department of Aging. This guide walks the whole path — licensing, staffing, getting paid, and getting clients — broken down so someone who has never done any of this can actually follow it.

Ohio changed the rules in 2022 — you now need a state license. Older “how to start home care in Ohio” articles say Ohio doesn't license non-medical home care. That is no longer true. ORC Chapter 3740 (effective Oct 1, 2022) requires an ODH Nonmedical Home Health Services License for agencies (and self-employed “nonagency providers” serving more than two clients who aren't family). Get the current requirements straight from ODH: LICCERT@odh.ohio.gov, 614-466-7713 — don't rely on out-of-date blog posts.

The scope line that decides whether you need the license. The license trigger is home health aide services — hands-on help with bathing, dressing, toileting, ambulation (walking), and feeding — plus personal care. If your business does ONLY companion sitting and homemaker chores (cooking, cleaning, errands, company) with no hands-on care, you MIGHT fall outside the trigger — but it's ambiguous, so confirm your exact service mix with ODH before assuming you're exempt. This guide covers the licensed Nonmedical path, which is what you need to serve most elder-care clients and to bill Medicaid.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened an agency — every time we name a thing (an agency, a form, a rule, an insurance type), we stop and break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with ODH, Ohio Medicaid, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, operating, paid agency. Each part below is one rung on the ladder.

1	Decide this is your business	A licensed non-medical in-home care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance & the surety bond (Part 3)
3	Get the ODH license	Nonmedical Home Health Services License, \$250 + \$20k bond (Part 4)
4	Set up your operation	Policies + your criminal-records-check policy (Parts 4–5)

5	Recruit & screen aides	Competency, BCI/FBI WebCheck, TB (Part 5)
6	Set up EVV & operations	Sandata EVV, policies, systems (Part 6)
7	Serve private-pay clients	Start earning while Medicaid enrollment proceeds (Part 8)
8	Enroll for Medicaid	PNM enrollment + Home Care Waiver / PASSPORT (Part 8)
9	Get your first clients	Discharge planners, the 12 AAAs, care managers (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. Get your ODH Nonmedical license (with the \$20,000 surety bond) and build your policies to Chapter 3740, then launch on private pay so you're earning while the months-long Medicaid enrollment runs in the background. Add Medicaid through the PNM module and the Home Care Waiver / PASSPORT — and remember the Area Agencies on Aging both certify you AND send you clients.

PART 1

What This Business Is

In short: An ODH-licensed Nonmedical Home Health Services agency — aides who help clients with daily living in their own homes. Agency-based, paid by private pay and Medicaid, and now a licensed business in Ohio.

Ohio (ORC Chapter 3740) licenses home health agencies by the type of service they provide. Your business provides non-medical services: home health aide help — hands-on bathing, dressing, grooming, toileting, help walking and transferring, meal preparation and feeding — plus homemaker tasks (light housekeeping, laundry, errands) that go along with personal care. Skilled nursing and therapy are the separate Skilled license and a different, bigger business.

Nonmedical vs. Skilled Home Health Services license

	Nonmedical (this guide)	Skilled
What it is	Home health aide + personal care	Nursing, therapy under a plan of care
ODH license	Nonmedical Home Health Services License	Skilled Home Health Services License
Fee / surety bond	\$250 + \$20,000 bond	\$250 + \$50,000 bond
Medications	Assistance/reminders only	Nurses administer & manage medications

This guide is for the Nonmedical license. You can add the Skilled license later if you decide to bring nursing and therapy in-house — but that's a separate application with its own clinical requirements.

The one clinical line to respect. Your non-medical aides may remind a client to take their own medication and hand them the bottle — but they may NOT administer medications by dose, give injections, or do nursing tasks. Those require the Skilled license and licensed nurses. Keep your scope clear in writing so no aide drifts across the line.

Key terms, in plain English. ODH = Ohio Dept. of Health (your licensor). ORC 3740 = the home-health licensing law. ODM = Ohio Dept. of Medicaid. ODA = Ohio Dept. of Aging. Home Care Waiver = the ODM personal-care waiver (ages birth–59). PASSPORT / Assisted Living Waiver = the ODA waivers (ages 60+). PNM = Provider Network Management (Ohio Medicaid's online enrollment system). BCI / WebCheck = the state fingerprint background-check system. EVV = Electronic Visit Verification (the clock-in/out system Medicaid requires; Ohio uses Sandata). AAA = Area Agency on Aging. NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Ohio has a large older population and, now that non-medical home care is licensed, a more professional market with room for reliable operators. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: ODH licenses you, ODM and ODA (with the Area Agencies on Aging) certify you for Medicaid and send you clients, the WebCheck program screens your staff, and Sandata verifies Medicaid visits.

ODH — Ohio Department of Health (your licensor)

ODH issues your Nonmedical Home Health Services License under ORC Chapter 3740, sets and enforces the requirements, and can take enforcement action against unlicensed operators. This is the office you call first. Licensure email LICCERT@odh.ohio.gov; phone 614-466-7713; odh.ohio.gov (search “home health agencies”).

ODM, ODA & the Area Agencies on Aging (Medicaid)

Ohio Medicaid pays for in-home personal care two ways: the Ohio Home Care Waiver (run by the Department of Medicaid, for people birth–59) and PASSPORT plus the Assisted Living Waiver (run by the Department of Aging, for people 60+). You enroll through Ohio Medicaid's PNM module; ODA certification adds a Provider Certification Wizard and an on-site pre-certification review by your local Area Agency on Aging. ODM provider line 800-686-1516; ODA 866-243-5678.

The Ohio Attorney General — WebCheck (background checks)

Ohio requires fingerprint-based BCI (Ohio Bureau of Criminal Investigation) criminal checks — plus an FBI check if the person lived outside Ohio in the last five years — done through the Attorney General's WebCheck program (877-224-0043). For the ODA waivers, several additional database checks apply (Part 5).

EVV — Electronic Visit Verification (Sandata)

For Medicaid personal-care and home-health visits, Ohio requires Electronic Visit Verification — an electronic record of who provided care, for whom, where, and when — using the state vendor Sandata. A claim that doesn't match a verified EVV visit won't be paid. Private-pay-only work is not subject to EVV.

The Long-Term Care Ombudsman & Adult Protective Services

The Ohio State Long-Term Care Ombudsman is a free, independent advocate for people receiving home care and long-term services — you give clients the ombudsman's contact information, and complaints can be investigated independently (1-800-282-1206, hosted regionally by the Area Agencies on Aging). You are also a mandated reporter: any suspected abuse, neglect, or exploitation of an adult goes to county Adult Protective Services immediately. Build both into your policies and your caregiver training.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Ohio you form the LLC at the Ohio Secretary of State — and one nice perk is that Ohio LLCs file NO annual report, so there's no yearly state filing to keep it active. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. Go to the Ohio Secretary of State.** File Articles of Organization (Form 610) online at OhioBusinessCentral (ohiosos.gov). The filing fee is \$99, one time.
- 2. Name the LLC and list a statutory agent.** The statutory (registered) agent has an Ohio street address and receives legal mail — this can be you, a partner, or a paid service (~\$50–150/yr).
- 3. Adopt an Operating Agreement.** Ohio doesn't file it, but your bank and any partners will want it — it sets out ownership percentages, who manages the company, and how money is split.
- 4. Register a trade name if you'll use one.** If your public brand differs from the LLC's legal name, file a Name Registration (Form 534A). Then register with the Ohio Department of Taxation and set up your Ohio BWC workers'-comp account (Ohio buys workers' comp from the state fund, not a private insurer).

Two Ohio simplifications. Ohio does NOT require newspaper publication to form an LLC, and Ohio LLCs file NO annual report — forming the entity is essentially a one-time \$99 filing.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. Even though private-pay work doesn't need it, you'll use the NPI to enroll with Ohio Medicaid and bill the waivers. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Ohio branches — Huntington (an Ohio-based bank), Fifth Third, KeyBank, PNC, or Chase, or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance & bonding — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so your risk profile is different from a store or office — you need coverage built for that, and the Nonmedical license itself requires a surety bond:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (a caregiver's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by an aide — general liability specifically EXCLUDES this, so it's a separate coverage	Yes — high exposure
Surety Bond (\$20,000)	Required by ORC 3740 for the Nonmedical license (a bond, not insurance — you buy it and pay a small annual premium)	Yes — required to be licensed
Non-Owned & Hired Auto	Your aides driving their own cars to clients or driving clients to appointments	Yes — critical for home care
Employee Dishonesty Bond	Theft from a client's home by an employee — families will ask if your caregivers are “bonded”	Yes — clients & payers expect it
Workers' Compensation (Ohio BWC)	Aide injuries (lifting and transferring is high-risk) — bought from the state fund	Required in Ohio for employees

Ohio workers' comp is a state fund. Ohio is a “monopolistic” state — you buy workers' compensation from the Ohio Bureau of Workers' Compensation (BWC), not a private insurer. Set up your BWC policy as soon as you have your first employee (bwc.ohio.gov).

Real places to get it

Buy through an independent agent or broker who writes home-care programs — they can package the liability, abuse, auto, and dishonesty coverages AND place your \$20,000 surety bond in one place. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, Nationwide (an Ohio-based insurer), and home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local Ohio agent, use the “Trusted Choice” Find-an-Agent directory at trustedchoice.com (filter for home care), or ask a franchise or peer. Confirm each carrier writes non-medical home care in Ohio before you rely on a quote.

What to ask the insurance agent

“Do you write Ohio Nonmedical Home Health agencies — which carriers, and what's the minimum premium?”

“Can you place the \$20,000 surety bond the license requires, and is abuse & molestation included or a separate sub-limit?”

“Do you include non-owned & hired auto (my aides drive their own cars) and an employee-dishonesty bond?”

“What coverage limits do the Ohio Medicaid waivers, the VA, or long-term-care insurers require to contract with me?”

“How do I set up my Ohio BWC workers'-comp account, and roughly what will it cost for my caregiver payroll?”

PART 4

Get Your ODH License — Step by Step

In short: Apply to ODH for the Nonmedical Home Health Services License with your fee, your \$20,000 bond, a fingerprint check, and your written policies. ODH suggests allowing about 90 days, so start early.

4.1 Before you apply — the checklist

- ☐ Your LLC is formed; you have your EIN, your Type 2 NPI, and an Ohio BWC workers'-comp account
- ☐ The \$20,000 surety bond is issued, and your liability + abuse + auto coverage is bound
- ☐ You've written a criminal-records-check policy (ODH requires this specific policy) and your full policy manual
- ☐ Owner/administrator fingerprint (BCI/FBI) checks are underway through WebCheck
- ☐ You've decided your service area (which Ohio counties you'll serve) and your office location

4.2 The application, step by step

- 1. Request the application from ODH.** Contact ODH's licensure program (LICCERT@odh.ohio.gov; 614-466-7713) for the current Nonmedical Home Health Services License application and instructions — the forms are updated periodically, so get the current version rather than an old copy.
- 2. Pay the \$250 license fee and post the \$20,000 bond.** The fee is non-refundable. The bond is a financial guarantee — you buy it through a surety/insurance agent and pay a small annual premium (often a few hundred dollars, depending on your credit).
- 3. Attach the required documents.** A fingerprint-based criminal-records check (under ORC 109.572), identity verification for the owner/administrator, your written criminal-records-check policy, a description of the services you'll provide, and your policies and procedures manual.
- 4. Submit and track your application.** ODH reviews it for completeness and may come back with questions — respond quickly, because the clock effectively restarts each time something is missing.
- 5. Get your license — then don't jump the gun.** ODH suggests roughly 90 days to process. Operating without a required license can be prosecuted, so do NOT provide hands-on personal care to clients until your license is in hand.

What this means for you: the two things that most often slow an Ohio application down are (1) an incomplete or missing criminal-records-check policy, and (2) a policy manual that doesn't cover everything Chapter 3740 expects. Build both to standard before you submit and you avoid the back-and-forth that turns 90 days into six months.

4.3 Timeline & what to do while you wait

Plan for about three months from a complete application to your license. Use that time productively: run your Medicaid (PNM) enrollment, recruit and screen your first aides, sign up for Sandata EVV, and line up private-pay referral sources so you can start serving private-pay clients the day the license issues — you don't have to wait for Medicaid to begin.

Ask before you file. Call ODH at 614-466-7713 to confirm the current application version, the exact list of required attachments, and whether any part of your intended service mix changes the licensure requirement. A complete first submission is the single biggest thing that saves you weeks.

PART 5

Your Aides — Competency, Training & Screening

In short: *Ohio's Medicaid aide standard is competency-based (not a fixed number of classroom hours), with RN supervision and layered background checks. Here's the whole system, one piece at a time.*

5.1 Aide competency & training (for Medicaid)

For the Ohio Home Care Waiver, an aide must be able to prove competency for the tasks they perform — here's what that actually requires:

- **A competency certificate:** the aide holds a certificate from an ODH-approved (or Medicare-approved) competency-evaluation or training-and-competency program. It's competency-based, not a fixed “60-hour course” — confirm the exact approved-program hours with your training provider.
- **Current first aid:** a hands-on first-aid certification (a course that includes a return demonstration, not just an online quiz).
- **Continuing education:** agency aides complete 8 in-service hours every 12 months, documented in the aide's file.
- **RN/LPN supervision:** for agency aides, a nurse makes a face-to-face supervisory home visit at least every 60 days while the aide is present with the client — plan for a nurse (employed or contracted) to do these.

What this means for you: even though you're a non-medical agency, serving Medicaid clients pulls a nurse into your model for the 60-day supervisory visits and for signing off on competency. Budget for a part-time or contracted RN/LPN from the point you start taking waiver clients.

5.2 Background checks — the exact system you must run

1. **Fingerprint through WebCheck.** Get a BCI (Ohio) criminal-records check on every owner and employee. Add an FBI check if the person has lived outside Ohio at any point in the last five years. Use an Ohio Attorney General WebCheck location (877-224-0043).
2. **Run the ODA database checks (for the 60+ waivers).** Ohio Administrative Code 173-9-03 adds several checks: the nurse aide registry, the abuser registries, the sex-offender registry, and the federal OIG exclusion list. Re-check on the required schedule.
3. **Apply your criminal-records-check policy.** ODH requires you to have a written policy listing disqualifying offenses and how you handle results — follow it consistently for every hire, and document your decision.
4. **Screen and document TB and identity.** Follow your infection-control policy for TB screening (confirm the current expectation with ODH), verify identity, and keep every dated result in the employee's file where a surveyor can find it.

5.3 Private-pay caregivers — set your own bar

For private-pay-only work there's no state-mandated aide training hour count, but you must still follow your license's criminal-records-check policy and screen every caregiver. Smart operators go further: a solid orientation, a skills check-off before an aide works alone, TB screening, and consistent scheduling. Treat caregivers as W-2 employees (you handle payroll, taxes, workers' comp, and supervision), because caregiver quality and consistency — the same aide showing up on time every time — is the single biggest driver of client satisfaction, referrals, and retention.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Getting Operational

In short: Your ODH policies are your operating backbone; add Sandata EVV for Medicaid visits and your scheduling/billing systems, and keep every file inspection-ready.

6.1 Your policies & procedures (the P&P Manual)

Build a policy-and-procedure manual to ORC Chapter 3740 and the Medicaid waiver rules. At minimum it covers: client rights and confidentiality; intake, assessment, and the plan of care; aide competency, training, and supervision; your criminal-records-check policy; complaint handling; incident and abuse/neglect reporting; emergency preparedness; and infection control. This manual is your operating backbone and the first thing a reviewer reads — a complete manual and matching forms is the single biggest time-saver at licensure and at any later survey.

6.2 Set up EVV (before your first Medicaid visit)

Electronic Visit Verification is mandatory for Medicaid personal-care and home-health visits and is the gate to getting paid.

- 1. Register for Sandata.** Ohio's EVV vendor is Sandata; onboard your agency, set up your aides, clients, and service authorizations.
- 2. Train your aides on clock-in/out.** Each visit records who provided care, for whom, where, and when — by phone, a mobile app, or a small device in the client's home.
- 3. Fix exceptions with visit maintenance.** When a visit doesn't capture cleanly, correct it promptly so the claim will pay.
- 4. Bill against verified visits.** A Medicaid claim must match a verified EVV visit — expect EVV compliance reviews, and keep your match rate high.

Private pay is EVV-free. EVV applies only to Medicaid visits. If you launch on private pay and long-term-care insurance, you don't need EVV until your first Medicaid client — one more reason private pay is the easier on-ramp.

6.3 Your systems

Pick a home-care software platform that handles scheduling, caregiver and client records, visit documentation, invoicing, and payroll — and that integrates with Sandata EVV for your Medicaid line. Getting this right early saves enormous administrative pain as you grow.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or via a waiver), you assess and set up the plan, an aide delivers the hours (with EVV and nurse oversight for Medicaid), you supervise, and you review and adjust. Here is the whole journey, stage by stage.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health or hospice partner, or an Area Agency on Aging / waiver case manager (Part 10). Someone calls asking whether you can staff care for a specific person.

What you do on that first call:

- Take the basics — who needs care, what tasks, how many hours a week, where, and when to start; confirm who pays (private, Home Care Waiver/PASSPORT, VA, or long-term-care insurance); and confirm the need is non-medical and within your scope.

Stage 2 — Assessment & the plan of care

Do an in-home assessment and build a plan of care: the specific tasks (bathing, dressing, meals, light housekeeping, transport, companionship), the schedule and hours, and any safety concerns.

What to review in the home:

- The client's mobility and transfer needs; the layout and any fall hazards; medications and who manages them; who else is involved (family, a case manager, a physician); and, for Medicaid clients, the waiver case manager's authorization, which sets the approved hours your plan must align to.

Stage 3 — Agreement & start of care

The client (or their representative) signs a service agreement covering the services, the schedule, the rate and payer, client rights, and how service can end. Match a screened, competent aide to the client — and make the first visit a well-introduced, supervised start, because the first impression drives retention.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The aide delivers the scheduled care; for Medicaid visits, each one is captured in Sandata EVV. Keep clear visit notes, and protect the client-aide match — sending the same familiar aide every time is what clients and families value most.

Stage 5 — Supervision & quality

For Medicaid clients, your RN/LPN makes the required 60-day supervisory visits. Across all clients, follow your supervision policy, monitor the aide's performance and the client's changing needs, run your quality program (satisfaction checks, complaint handling, incident reports), and report any suspected abuse, neglect, or exploitation immediately.

Stage 6 — Changes & reassessment

As the client's needs change, update the plan of care; for waiver clients, coordinate reassessments and new authorizations with the case manager. Keep authorizations current — delivering hours that aren't authorized doesn't get paid.

Stage 7 — Discharge & transition

When care ends — the client recovers, moves to a higher level of care, or transitions to hospice — follow your discharge policy, give proper notice, coordinate with the family and any waiver, and document the transition. A good discharge and a follow-up call turn former clients and their families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid, VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're licensed; add Medicaid through the Home Care Waiver (ODM) and PASSPORT / Assisted Living (ODA). Personal care is billed by the unit/hour.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start the day you're licensed; highest rate; no EVV or enrollment needed
Medicaid — waivers	Ohio Home Care Waiver (ODM) + PASSPORT/AL (ODA)	The big, steady market; requires PNM enrollment, ODA certification & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or the insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 2, Optum)

8.1 Private pay — your launch market

The day your license issues you can serve private-pay clients — families paying out of pocket, typically billed by the hour with a visit minimum (often 3–4 hours). Pull the current Ohio private-pay non-medical rate from a CareScout/Genworth cost-of-care report, and set your rate above your fully-loaded aide cost (wage + payroll taxes + workers' comp + overhead). No EVV, no Medicaid enrollment, no waiting — just a clear service agreement and good caregivers. Long-term-care insurance is part of this stream: many clients have an LTC policy that reimburses in-home personal care once ADL or cognitive benefit triggers are met, and most policies require a licensed agency — which you now are; you bill the family or the insurer and help with the claim.

8.2 Medicaid — the waivers (the bigger market)

Ohio pays for in-home personal care through the Ohio Home Care Waiver (ODM, for people birth–59 at nursing-facility level of care) and PASSPORT plus the Assisted Living Waiver (ODA, for people 60+). Much of it is delivered through Ohio Medicaid managed care. Here's how to get set up to bill:

- 1. Hold your ODH Nonmedical license first (Part 4).** You can't enroll to bill Medicaid without it.
- 2. Enroll through the PNM module.** Apply through Ohio Medicaid's Provider Network Management module with an OH|ID account and your Type 2 NPI (ODM provider line 800-686-1516).
- 3. Complete ODA certification (for the 60+ waivers).** Use the same PNM application, then finish ODA's Provider Certification Wizard within 10 days. Your local Area Agency on Aging does an on-site pre-certification review (within 60 days) before ODA certifies and contracts you.
- 4. Set up Sandata EVV (Part 6).** Do this before you deliver a single Medicaid visit, or the claims won't pay.

Personal care is billed per 15-minute unit; the exact rate lives in the ODM Home Care Waiver fee schedule and the ODA rate rules — pull the current per-unit figure there before you rely on it, and confirm the timely-filing deadline.

How you actually bill (the claims cycle)

- 1. Verify eligibility before and during the stay.** Check the member's Medicaid enrollment and waiver each month — enrollment can change, and delivering care to someone who's lost coverage means you don't get paid.
- 2. Confirm the authorization.** The waiver case manager's service plan authorizes a specific service and a number of units/hours — you can only bill what's authorized, so watch the dates and the amount.
- 3. Capture every visit in Sandata EVV.** The visit must be verified before the claim will pay (Part 6).
- 4. Submit the claim.** Bill electronically through Ohio Medicaid (via the PNM/claims system) or the managed-care plan, using your NPI and the authorized service code and units.
- 5. Watch timely filing and reconcile.** File within the deadline (confirm the current window), then reconcile every remittance so nothing slips through unpaid.

Ask ODM / your AAA:

- The current per-unit rate for personal care; how a reassessment changes the authorization and rate; the claims portal and the timely-filing window; and how managed-care vs. fee-for-service billing works for your specific members.

8.3 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's non-medical in-home programs — aides work for an agency that holds a VA contract or is in the VA Community Care Network. Ohio is in Region 2, administered by Optum. This is your entry point to serve veterans.
- **Aid & Attendance:** an increased VA pension for eligible wartime veterans and survivors, which the veteran can apply toward your private-pay care — effectively making your service affordable. Verify current amounts on VA.gov.

PART 9

What It Costs & How Long It Takes

In short: Budget the license and surety bond, insurance, aide training, EVV, payroll float, and (optionally) accreditation. Launch on private pay, then add Medicaid.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
ODH Nonmedical license	Initial license fee	\$250
Surety bond	Required for the Nonmedical license	\$20,000 bond (you pay a small annual premium)
Business setup	SOS Form 610 \$99 (no annual report), EIN free, NPI free, legal	~\$99 + legal
Insurance & BWC	Liability + abuse + non-owned auto + dishonesty + Ohio BWC workers' comp	Get quotes — annual
Aide training	Competency program + first aid per aide	Per aide
EVV & software	Sandata + a scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay aides weekly before Medicaid/insurers pay you	Working capital — plan for it
Accreditation (optional)	ACHC / CHAP / Joint Commission	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. Home care is payroll-heavy: you pay aides weekly, but Medicaid and insurers pay you weeks later. Private-pay clients (billed in advance or on short terms) smooth this out — line up a business line of credit before you scale Medicaid hours.

9.2 If you pursue accreditation

- **Why do it:** some Medicaid managed-care plans, VA contracts, long-term-care insurers, and franchise brands prefer or require accreditation, it builds credibility with hospital referral sources, and it's the on-ramp if you later add Medicare-certified skilled care.
- **Who accredits home care:** ACHC, CHAP, and The Joint Commission all accredit home care; each runs a survey and a 3-year term.
- **When to do it:** most non-medical agencies launch on their state license/registration and add accreditation in year two, once volume justifies the cost (commonly \$7,000–\$14,500 over a 3-year cycle plus staff time).

9.3 Timeline

Weeks	Entity, bond & policies	Form the LLC, get the \$20k bond, write your P&P to ORC 3740
Weeks	Insurance & BWC	Bind coverage; set up your Ohio BWC workers'-comp account
~90 days	ODH application → license	Apply + \$250 + bond + fingerprints; ODH processes
Right away	Start private pay	Serve private-pay & LTC-insurance clients on your license
Parallel	Enroll for Medicaid	PNM enrollment + ODA certification + Sandata EVV
Ongoing	Referrals & census	Discharge planners, AAAs, waiver case managers

9.4 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$30/hour (set yours from your local market and what referral sources see).

- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the 12 Area Agencies on Aging, waiver case managers, care managers, and senior communities. Build these relationships before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the single highest-volume source for post-hospital home support — introduce yourself, leave a one-pager, and make it effortless to refer to you and start same-day.
- **Home-health & hospice partners:** they refer the non-skilled companion and personal-care hours their benefit doesn't cover; build reciprocal partnerships (you refer skilled needs to them).
- **Area Agencies on Aging (12 in Ohio):** these are doubly important in Ohio — the AAA authorizes PASSPORT, does your on-site certification review, AND routes waiver clients to certified providers. Build a strong relationship with each AAA in your service area. Statewide line 866-243-5678; aging.ohio.gov.
- **Waiver case managers:** ODM Home Care Waiver and ODA case managers route member hours to certified providers — stay in close contact and be responsive.
- **Geriatric care managers & senior living communities:** private Aging Life Care professionals place clients with vetted agencies, and assisted-living and independent-living communities refer supplemental one-on-one care for residents.
- **Ohio 2-1-1:** list your services with Ohio 2-1-1 (dial 211) — a free statewide referral channel families call when they need help at home.

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the bond & staff

Who	Contact	What to verify / ask
ODH — Home Health Agency Licensure	614-466-7713 · LICCERT@odh.ohio.gov	The Nonmedical license (\$250 + \$20k bond); the application version; the ~90-day timeline; your scope trigger
Ohio AG — WebCheck	877-224-0043 · ohioattorneygeneral.gov	BCI/FBI fingerprint checks and the disqualifying offenses
Ohio BWC (workers' comp)	bwc.ohio.gov	Setting up your state-fund workers'-comp policy
Surety bond agent	a bond/insurance agent	The \$20,000 surety bond for the Nonmedical license

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Ohio Secretary of State	ohiosos.gov	Forming your LLC (Form 610, \$99); no annual report; trade name (534A)
IRS	irs.gov (EIN Assistant)	Getting your EIN (free)
NPPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI and the right taxonomy
Insurance agent (home-care program)	trustedchoice.com; carriers in Part 3.5	Liability + abuse + non-owned auto + the \$20k bond + BWC

Funding, EVV & referrals

Who	Contact	What to verify / ask
Ohio Medicaid — PNM enrollment	800-686-1516 · medicaid.ohio.gov	Enrolling your NPI via PNM; the Home Care Waiver; the current per-unit rate; timely filing
Ohio Dept. of Aging — certification	866-243-5678 · aging.ohio.gov	ODA certification (the Wizard + the AAA on-site review); PASSPORT / Assisted Living
Ohio Medicaid — EVV / Sandata	medicaid.ohio.gov (EVV)	Sandata onboarding and EVV compliance
VA Community Care / Optum (Region 2)	va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (12)	866-243-5678 · aging.ohio.gov	They certify you AND route waiver clients — your most important relationship
Ohio 2-1-1	dial 211	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; payer requirements
Hospital / SNF discharge planners	your local hospitals & SNFs	The highest-volume referral source

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

You can do this

Opening an Ohio non-medical in-home care agency now means getting a real state license — but it's a defined, learnable path, and a solid Medicaid market (the Home Care Waiver and PASSPORT) runs alongside private pay. Take it one rung at a time: form the business, get the \$20,000 bond and the ODH Nonmedical license, recruit and screen great aides, launch on private pay, enroll with Ohio Medicaid through PNM and get ODA-certified, and build referral relationships with the Area Agencies on Aging and discharge planners. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in ORC Chapter 3740 and current Ohio Medicaid information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with ODH, Ohio Medicaid, and qualified counsel before you rely on it. In particular, confirm your service-scope licensure trigger, the current per-unit rate, and your EVV setup before you rely on them.